

إسم المريض ربيعاً : آمنه علي نصرالدين +++
المحافظة : الجيزة الحي / المدينة : كداسه
رقم البطاقة : 25803112101462 رقم التليفون : 01012171821
العمر(تاريخ الميلاد) : 11-03-1958 المهنة : ربه منزل
العنوان : ++++ تاريخ تسجيل الإستمارة : 05-10-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: امنه علي نصرالدين +++ National ID: 25803112101462	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

امنہ
على نصر الدين
١٠ منشأة البكرى
مركز كرايمه - الجيزة

١٩٥٨/٠٣/٢١ ٢٥٨٠٣١١٢١٠١٤٦٢
I01918391

٢٠٢٠/٠٩ ٢٥٨٠٣١١٢١٠١٤٦٢

ربه منزل
أنثى مسلمة أرملة

٢٠٢٧/٠٩/١٢ البطاقة سارية حتى ١





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name: أمينة علي محمد الدين

Sex:

3- Medical numbers:
Female

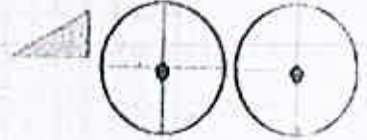
83118115

Old Med:

1936/2022

2-ID numbers: 25803112101462

Patient Age: 65 Y



Family History

Phone

01012171821

BMI

REASON OF MDT POST NACT

marital status WIDOW

NRS Score

GROUP

Date of MDT 03/09/2023

Co morbidity HTN

Menopausal status

Clinical data

Breast Imaging :

Mets Workup :
FREE LN (0/6)

BX:
IDC
G2

ER:
6/8

PR:
3/8

Her 2 Neu :
+VE

SISH :

KI67 :
15

T
2

N
1

M

Sonomammography

PATIENT HISTORY

PT FINISHED 6 TCHP NACT----LT BCS ---- YPTO YONO
STARTED AI FEMARA + CONTINUE HP TILL MDT
LT BCS :PCR

MDT OUTCOME & PLAN

FOR CONTINUE HERCEPTIN 600MG S.C FOR 1YEAR
PORT
ARAMEDIX FOR 7YRS
PROLIA/6MS

SIGNATURE

MEDICAL ONCOLOGY

9/13/2023

RADIOLOGY

PATHOLOGY

Nutrition

SURGERY

Psychiatric



Patient Name: Amna Ali Nasr Al Din	Date : 07/01/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 64 Year(s)
Medical ID: 1936/2022	Pat. ID: 11840

**Combined bilateral breast mammography and complementary
ultrasound**

Indication:

Left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Regressive course regarding left breast UIQ irregular spiculated high density masses.
- Complementary ultrasound showed left breast UIQ irregular spiculated hypoechoic masses, with internal vascularity on color Doppler application, seen at 9 o'clock, measured 1 x 1.5 cm compared to 1.5 x 1.7 cm, smaller adjacent lesion is also noted measures 0.5 cm, consistent to the palpable lump.
- No right breast suspicious cystic or solid mass lesions.
- Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Nonspecific right axillary LN.
- Regressive course regarding pathological left axillary lymph nodes.

Opinion:

- Follow up post NACTH showing partial therapeutic response (BIRADS 6).
- Right breast normal study (BIRADS 1)

Signature Dr. Aya Deabes

07/01/2023 1:37 PM

Request No.	13002054
Request Date	09/05/2022
Verification Date	21/05/2022
Financial Category	اورام دار السلام - مہاجر
Department	Out Patient
IHC No.	
Registered by	Ali mostafa

Clinical Data: Left breast 11 o'clock malignant looking mass.

Tru cut biopsy, collected at 9/5/2022, at 11:40 AM.

Gross Examination:
Multiple grayish white and fatty tissue cores, up to 2.5 cm, totally submitted.
Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER,
PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): II (3+3+2)

ER: Strong nuclear reaction in 80% of tumor cells (Allred score 5/8)
 PR: Moderate nuclear reaction in 10% of tumor cells (Allred score 3/8)
 Her-2: strong complete membrane reaction in >10% of tumor cells (score 3+)
 Ki 67: 15 % nuclear reaction

Left breast mass; Tru cut biopsy:

- Invasive duct carcinoma, grade II
- ER : Positive (8/8)
- PR : Positive (3/8)
- HER2 : Positive (score 3+)
- Ki67: 15%

Prof.Dr. Aml Hareedy

Printing Date : 25/05/2022 09:42:58

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Printed By: marwa mohamed youss



Patient Name : المرضى / Patient Name	Request No. : 13001714
Patient No. : 25803112101462	Request Date : 12/02/2023
Pain No. : 435/2023	Verification Date : 21/02/2023
Sex : Female	Financial Category : مختبر - مختبر
Age : 60 Y	Department : Out Patient
Referring Doc :	Registered by : All mostafa

Pathology Report

Clinical Data:
Left breast cancer , post NACT

Nature of the specimen:

Left WLE & SLNB specimens , collected at 12/2/23

Gross Examination:

Two specimens were received .

SLNB - Fibrofatally tissue piece measured 5x3 cm , enclosing 6 nodules up to 1.5x1cm

Left WLE specimen measured 8x6x1.5 cm with covering skin ellipse measured 8x1.5 cm.

• Tumor extent

• Number. An ill defined rubbery to firm greyish white tumor bed site

• Size: 1.7x1.5 cm

• Distance from margins: 0.8 cm from superior margin (least margin)

• Frozen section:

• Free SLNs (0/5) and one suspicious lymph node

• Free resected margins

Microscopic examination:

• Total tumor regression in a previously diagnosed invasive carcinoma

• Angiolymphatic invasion: Not detected

• Ductal carcinoma in situ: Not detected

• Lymph node involvement: Free lymph nodes (0/6)

• Resection Margins: Free all resected margins

• Histologic response to therapy: Pathologic complete response

• Additional pathologic findings: Focal microcalcifications

Diagnosis:

Verified by :

walaa ghoniem

Printing Date : 26/02/2023 10:44:40

Page 1 of 2

Printing By : marwa mohamed youss



Patient Name : **د. علي محمد علي** / Dr. Ali Mohamed Ali

Patient No. : **25803112101462**

Path No. : **435/2023**

Sex : **Female**

Age : **60 Y**

Referring Doc :

Pathology Report

Left breast cancer post NACT , WLE & SLNB specimens :

-Pathologic complete response in a previously diagnosed carcinoma

-Free all resected margins

-Free lymph nodes (0/6)

-Stage : ypT0 , ypN0

Prof Gamal Abdellaty

Dr Esraa Magdy

Verified by :
walea ghomem

Printing Date : 26/02/2023 10:44:40

Page 2 of 2

Printing By : marwa mohamed yous

Patient Name: Amna Ali Nasr Al Din	Date : 10/06/2024
Exam : MAMMOGRAPHY BOTH	Age : 66 Year(s)
Medical ID: 1936/2022	Pat. ID: 11840

**BILATERAL BREAST MAMMOGRAPHY AND HIGH
RESOLUTION ULTRASOUND**

Indication:

breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Bilateral benign looking vascular calcifications.

Left breast:

- Operative bed parenchymal distortion and scarring with no sizable masses
- Diffuse left breast skin thickening.
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Right Breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post left BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Signature Dr. Mohamed Mohsen

10/06/2024 4:52 PM

ADULTISLICE NON CONTRAST CT SCAN OF THE CHEST:

FINDINGS:

- Left breast overlying skin thickening.
- Left upper lobar subpleural reticulations and honeycombing likely post therapeutic sequel.
- No pathologically enlarged axillary lymph nodes.
- Few bilateral tiny pulmonary nodules the largest is seen at lateral segment of middle lung lobe measuring 2 mm.
- Prominent relatively hyperdense mediastinal lymph nodes in short axis.
- No plural or pericardial collection.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Moderate cardiomegaly.
- No pericardial effusion detected.
- Left thyroid gland hypodense nodule.

❖ Status follow up showing the above described changes.

Dr. Nourhan M/



NUCLEAR MEDICINE UNIT

Patient Name: Amna Ali NasrEldin

Date: 18/7/2022

وحدة الطب النووي

^{99m}Tc-MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No definite hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- Degenerative changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are seen

Conclusion:

- *The surveyed skeleton is free of any osseous deposits.*

Thanks for reference

Nuclear Medicine Unit

DR/Sherien Mahdy, MD